WHO

Oakridge JMUN 2026

World Health Organization

Agenda: Addressing Mental Health and Addiction Among Young People

CAR KIA
Jubilee Hills | HiTech City | Financial District

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A Quick Note Before You Start

This guide talks about mental health conditions, drugs, alcohol and, in a few places, suicide. All of it is written for learning and debate. If any part of it feels heavy or reminds you of something in your own life, that is completely okay. Please talk to a parent, a teacher, your school counsellor or any adult you trust. Asking for help is a strength, not a weakness. The Executive Board is also always happy to talk to you during the conference.

1. Letter from the Executive Board

Dear Delegates,

Welcome to the World Health Organization at OakJMUN!

Let us begin with a small confession. When we first read this agenda, all three of us did the same thing. We picked up our phones and checked our screen time. We will not be sharing those numbers with you. Some secrets are protected under international law. (They are not. But please let us have this one.)

That moment taught us something, though. This agenda is not about some faraway problem in some faraway place. It is about people your age and ours. It is about the friend who suddenly stops talking at lunch, the classmate who says they "just can't stop" playing a game, and the online advertisements that make vapes look like candy. You already know more about this topic than you think. Our job in this committee is to turn that everyday knowledge into real, thoughtful solutions.

Now, the honest part. This is a serious topic. Mental health and addiction affect real families, and some of the numbers in this guide are not easy to read. So we ask one thing from every delegate: be kind. You can disagree with another country's policy as strongly as you like, but always respect the people behind the statistics. A good WHO delegate argues hard about ideas and stays gentle with people.

You might be wondering what makes a delegate stand out. It is not the person with the longest speech or the fanciest vocabulary. It is the person who knows their country's position, listens to others, asks good questions, and helps the committee find common ground. If you have done your research, you are already halfway there. If you have not, well, you are reading this guide, so you are already on your way. We are proud of you.

A few things to keep in mind:

- This guide is a **starting point**, not a script. Use it to understand the topic, then dig into what *your* country thinks and does.
- Every fact here comes from an official source, and each one has a footnote. Please follow the same habit in your speeches and position papers.
- Nobody expects you to be a doctor or a scientist. We expect you to be curious, prepared and respectful.

- If you are nervous about speaking, that is normal. Every great speaker in this room was once a nervous first timer. Raise your placard anyway. We will be cheering for you (silently, because of the rules of procedure).

If you have any questions about the agenda, the rules, or anything else, please reach out to us. There is no such thing as a silly question. There is only the question you did not ask and then worried about for three days.

We cannot wait to meet you, hear your ideas, and watch you change the way the world thinks about young people's health. Charge your laptops, sharpen your arguments, and maybe, just maybe, put your phone down for an hour or two. For research purposes.

Warm regards,

Syed Hudaifah
Chairperson

Pratheek Konda
Vice Chairperson

Archith Ragi
Rapporteur

2. MUN Rules Of Procedure (ROP)

1. Roll Call

- The Chair calls on each country to declare if they are "Present" or "Present and Voting." It refers to the voting procedure on the Draft Resolution, where a delegate can vote "Yes", "No", or opt to "Abstain".
- If "Present and Voting," the delegate cannot abstain from voting. If the delegate chooses to respond with "Present", they may answer with "Present and Voting" on subsequent days, but this is not permissible for "Present and Voting" as this reply must be used consistently from that point forth.

2. Setting the Agenda

- Delegates motion to set the agenda, followed by a formal debate on which topic to discuss first. This motion requires a simple majority to pass.

3. Speakers' List

- After passing a motion to enter formal debate, delegates are placed on a speakers' list to make formal speeches. Time limits are set, and the Chair calls on delegates to speak in order.
- A delegate must raise their placard if they wish to add their name to this list. They can be on this list only once on any occasion.

4. Points

- **Point of Order:** To correct a procedural error by the Chair.
- **Point of Personal Privilege:** For personal discomfort (e.g., if the room is too cold, or if a fellow delegate cannot be heard).
- **Point of Inquiry:** To ask questions about discussion or statements in committee.
- **Point of Parliamentary Inquiry:** To ask questions with regard to the Rules of Procedure.

5. Motions

- **Motion to Suspend the Meeting:** To break for caucuses or adjourn the session.
- **Motion to Close Debate:** Ends the discussion on a topic and moves to voting. Requires a two-thirds majority.
- **Motion to Table the Topic:** Postpones debate on a topic. Requires a majority vote.

6. Yields

- **Yield to the Executive Board:** A delegate yields their remaining time from their speech to the Executive Board. It is to be used as they see fit.
- **Yield to Another Delegate:** A delegate yields their remaining time for another delegate to speak. (This delegate's permission must be sought beforehand.)
- **Yield to Points of Information:** A delegate yields their remaining time to Points of Information to the committee.
- **Yield to Comments:** A delegate yields their remaining time to comments from the committee.

7. Caucuses

- **Moderated Caucus:** The Chair controls the speaking order with shorter, focused speeches. The topic selected is a narrowed version of the agenda, chosen by the delegate who proposed the moderated caucus.
- **Unmoderated Caucus:** Informal discussion among delegates not under the moderation of the Executive Board.

8. Draft Resolutions and Amendments

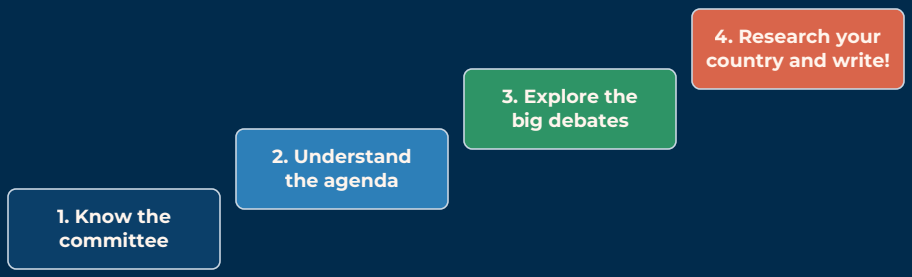
- Delegates collaborate to create draft resolutions, which are formal solutions to the discussed issue.
- **Friendly Amendments:** Accepted by all sponsors without a vote.
- **Unfriendly Amendments:** Need to be voted on to be added.

9. Voting

- **Procedural Votes:** Includes motions and amendments; all members must vote.
- **Substantive Votes:** For resolutions and unfriendly amendments; only "Present and Voting" delegates may vote, and abstentions are allowed.

3. How to Use This Guide

This guide is organised like a staircase. Each section builds on the one before it.



- **Blue boxes** contain key facts you can use in speeches.
- **Yellow boxes** contain "Think about it" questions to help you prepare.
- **Green boxes** contain notes about wellbeing and care.
- **Footnotes** at the bottom of each page tell you exactly where a fact came from. Click on the link to read the original document.

A Word About Numbers

Health statistics are *estimates*. Different reports use different years, different age groups and different methods. For example, one report may count 10 to 19 year olds while another counts 13 to 15 year olds. When you use a number in committee, always say **who** was counted, **when**, and **who** published it. That is what makes a delegate sound trustworthy.

4. About the Committee: The World Health Organization

4.1 A Short History

After the Second World War, countries realised that diseases do not stop at borders and that health needed international cooperation. In 1946, at the International Health Conference in New York, countries adopted the **Constitution of the World Health Organization**. It came into force on **7 April 1948**, which is why 7 April is celebrated every year as World Health Day.[1]

WHO is a **specialised agency of the United Nations**. Its headquarters are in Geneva, Switzerland, and it works through six regional offices covering Africa, the Americas, South-East Asia, Europe, the Eastern Mediterranean and the Western Pacific.[2]

What Does "Health" Mean to WHO?

The very first lines of the WHO Constitution define health as "a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity."[3]

Notice the word **mental**. Since 1946, WHO has treated mental health as part of health itself, not as a separate or less important issue.

4.2 How WHO Is Organised

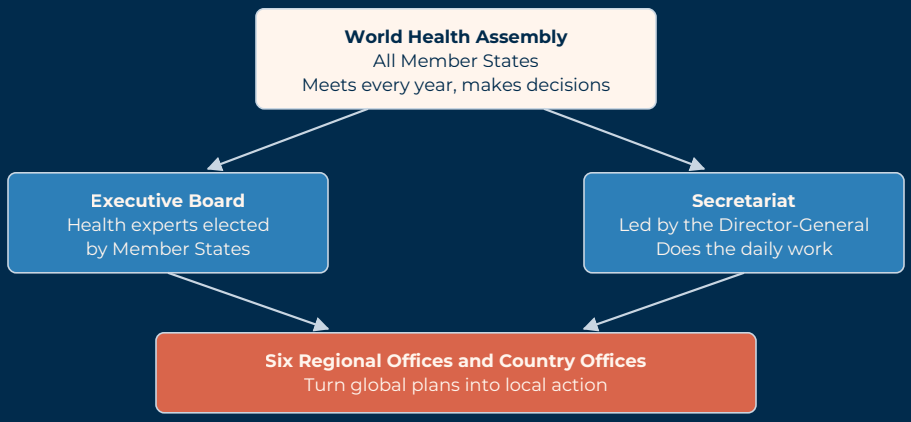


Figure 1: The main parts of WHO, as set out in its Constitution. Diagram made for this guide.

1. World Health Organization, Constitution of the World Health Organization (adopted 22 July 1946, entered into force 7 April 1948). Available at: https://www.who.int/about/governance/constitution
2. World Health Organization, regional websites listed on the official WHO homepage. Available at: https://www.who.int/
3. Constitution of the World Health Organization, Preamble. Available at: https://www.who.int/about/governance/constitution

4.3 The Mandate of WHO

A **mandate** is the official job description of an organisation. It tells us what WHO is allowed and expected to do. According to Article 1 of its Constitution, WHO's objective is "the attainment by all peoples of the highest possible level of health."[4]

Article 2 lists WHO's functions. The ones most relevant to our agenda are:[5]

- acting as the **directing and coordinating authority** on international health work;
- helping governments **strengthen their health services** when they ask for support;
- proposing **conventions, agreements and regulations**, and making **recommendations** on international health matters;
- **fostering activities in the field of mental health**, especially those affecting the harmony of human relations;
- promoting and conducting **research** in health;
- helping to develop an **informed public opinion** on health among all peoples.

What WHO Can and Cannot Do

WHO can: set global guidelines, collect and publish data, give technical help to countries, run joint programmes with other UN agencies, and, through the World Health Assembly, adopt conventions and regulations (Articles 19 and 21 of the Constitution).[6]

WHO cannot: pass laws inside a country, send police, or force a government to change its policies. Most of what WHO produces are *recommendations*. Countries decide for themselves whether and how to follow them.

Why this matters for you: a realistic WHO resolution *recommends, encourages, invites* and *requests*. It does not *order* countries around. Keep your clauses within the mandate!

4. Constitution of the World Health Organization, Article 1. Available at: https://www.who.int/about/governance/constitution
5. Constitution of the World Health Organization, Article 2. Available at: https://www.who.int/about/governance/constitution
6. Constitution of the World Health Organization, Articles 19, 21 and 23. Available at: https://www.who.int/about/governance/constitution

A Good Example of WHO's Power: A Real Treaty

The **WHO Framework Convention on Tobacco Control (FCTC)** was adopted in 2003 and entered into force in 2005. It was the first treaty negotiated under WHO's authority, and it commits its Parties to measures that reduce tobacco demand and supply.[7] It is directly linked to our agenda because many young people start using tobacco and nicotine products during adolescence.

Membership Update for Delegates

WHO has stated that in 2025 the UN Secretary-General circulated a notification from the United States of its intention to withdraw from WHO, with a stated effective date of 22 January 2026, and that this notification raises issues for WHO's governing bodies to consider.[8] Membership questions can change, so please check the latest official position of your assigned country before the conference.

5. Introduction to the Agenda

5.1 What Is This Agenda Really Asking?

Our agenda, "**Addressing mental health and addiction among young people**", asks the committee three big questions:

1. How can countries **protect and promote** the mental health of young people before problems begin?
2. How can countries **prevent** young people from starting harmful use of alcohol, tobacco, nicotine and drugs, and from developing unhealthy patterns with gaming and social media?
3. How can young people who are already struggling **get help** that is safe, affordable, respectful and easy to reach?

7. WHO Framework Convention on Tobacco Control, official website. Available at: https://fctc.who.int/

8. World Health Organization, WHO statement on notification of withdrawal of the United States (24 January 2026). Available at: https://www.who.int/news/item/24-01-2026-who-statement-on-notification-of-withdrawal-of-the-united-states

5.2 Key Words You Need to Know

Adolescent	A person aged 10 to 19. WHO notes that about one in six people in the world is in this age group.[9]
Mental health	How we think, feel, cope with stress, relate to others and make choices. Everyone has mental health, just like everyone has physical health.
Mental health condition (or disorder)	A health condition that significantly affects how a person feels, thinks or behaves, such as depression, anxiety or ADHD. These are real health conditions and can be treated.
Substance use	Using alcohol, tobacco, nicotine (including e-cigarettes or "vapes"), cannabis or other drugs.
Addiction or dependence	When a person finds it very hard to control their use of a substance or an activity, keeps doing it even when it causes harm, and gives it more and more priority over other parts of life.
Behavioural addiction	Addiction-like patterns linked to an activity rather than a substance. WHO recognises gaming disorder in its official list of diseases (ICD-11).[10]
Stigma	Negative attitudes or shame attached to a condition. WHO notes that stigma affects how ready adolescents are to seek help.[11]
Risk factor	Something that makes a problem more likely, for example bullying or violence.
Protective factor	Something that makes a problem less likely, for example supportive family, friends and schools.
Treatment gap	The difference between the number of people who need care and the number who actually receive it.

9. World Health Organization, Mental health of adolescents, fact sheet (1 September 2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

10. World Health Organization, Gaming disorder: questions and answers. Available at: https://www.who.int/standards/classifications/frequently-asked-questions/gaming-disorder

11. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

5.3 Why This Agenda Matters Right Now



1 in 7 adolescents (10 to 19 years)
experiences a mental disorder

Figure 2: Based on the WHO fact sheet "Mental health of adolescents" (2025). Pictogram made for this guide.

The Big Picture in Numbers

- Globally, about **one in seven (14.3%)** 10 to 19 year olds experiences a mental health condition, yet most of these conditions remain unrecognised and untreated.[12]
- Mental disorders account for about **15%** of the global burden of disease in this age group.[13]
- **More than 1 billion people** of all ages live with a mental health condition worldwide.[14]
- Suicide is the **third leading cause of death** among people aged 15 to 29.[15]
- Globally, **23.5%** of all 15 to 19 year olds were current drinkers of alcohol in 2019.[16]
- At least **15 million children aged 13 to 15** already use e-cigarettes. In countries with data, children are on average **nine times** more likely than adults to vape.[17]

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12. WHO, Mental health of adolescents, fact sheet (2025), citing the Global Burden of Disease 2021 database. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

13. Ibid. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

14. World Health Organization, Over a billion people living with mental health conditions: services require urgent scale-up, news release (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up

15. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

16. World Health Organization, Over 3 million annual deaths due to alcohol and drug use, majority among men, news release (25 June 2024). Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

17. World Health Organization, WHO tobacco trends report: 1 in 5 adults still addicted to tobacco, news release (6 October 2025). Available at: https://www.who.int/news/item/06-10-2025-who-tobacco-trends-report-1-in-5-adults-still-addicted-to-tobacco

5.4 Why Mental Health and Addiction Are Discussed Together

You might ask: why put mental health and addiction in one agenda? The answer is that they are closely connected. WHO explains that many risk-taking behaviours, such as substance use, start during adolescence, and that risk-taking can be "an unhelpful strategy to cope with emotional difficulties."[18] WHO also notes that eating disorders often occur alongside depression, anxiety and substance use disorders, and that harmful use of alcohol is among the risk factors for suicide.[19]

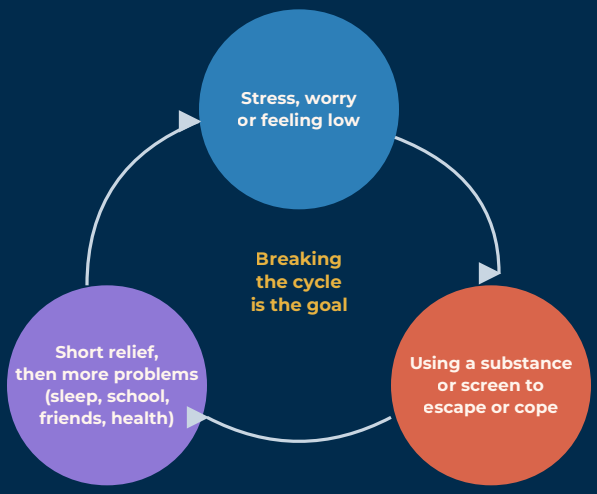


Figure 3: A simple way to picture how emotional difficulties and risky coping can feed each other, based on WHO's description of risk-taking as an unhelpful coping strategy. Diagram made for this guide; real lives are more complicated than any one diagram.

6. Understanding the Agenda, Part A: Mental Health of Young People

6.1 Why Is Adolescence Such an Important Time?

WHO describes adolescence as "a unique and formative time."[20] Young people go through big physical, emotional and social changes. It is also the time when many healthy habits are built, such as good sleep, regular exercise, problem-solving skills, getting along with others and learning to manage emotions. Supportive

18. WHO, Mental health of adolescents, fact sheet (2025), section on risk-taking behaviours. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

19. Ibid., sections on eating disorders and on suicide and self-harm. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

20. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

environments at home, at school and in the community are important for all of these.[21]

WHO also warns that failing to address mental health conditions in adolescence has consequences that "extend to adulthood", affecting both physical and mental health and limiting opportunities later in life.[22] In simple words: helping a 13 year old today also helps the 30 year old they will become.

6.2 What Affects a Young Person's Mental Health?

WHO explains that "the more risk factors adolescents are exposed to, the greater the potential impact on their mental health."[23] The diagram below groups the factors WHO mentions into layers, from the person at the centre to the wider world around them.

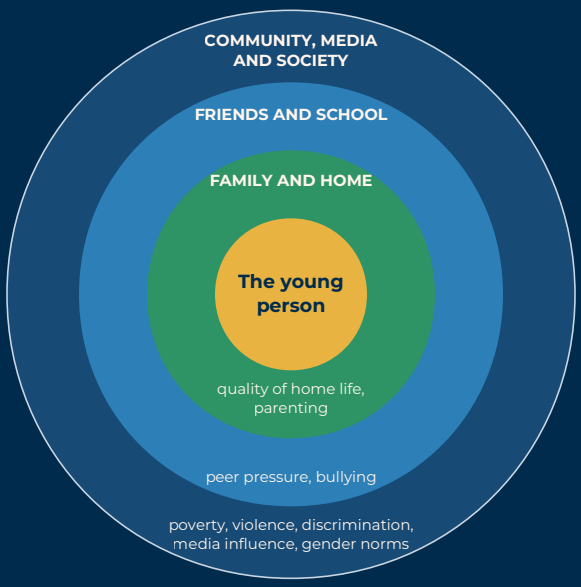


Figure 4: Layers of influence on adolescent mental health. Factors are taken from the WHO fact sheet "Mental health of adolescents" (2025). Diagram made for this guide.

Some of the risk factors WHO lists are: exposure to adversity, pressure to fit in with peers, exploring one's identity, media influence, gender norms, violence (especially

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21. Ibid., section on mental health determinants. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

22. Ibid., key facts. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

23. Ibid., section on mental health determinants. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

sexual violence and bullying), harsh parenting and serious socioeconomic problems.[24]

Who May Need Extra Support?

WHO identifies groups of adolescents at greater risk because of their living conditions, stigma, discrimination, exclusion or lack of access to services. These include adolescents living in humanitarian and fragile settings; those with chronic illness, autism, an intellectual disability or another neurological condition; pregnant adolescents and adolescent parents; those in early or forced marriages; orphans; and adolescents from minority ethnic or sexual backgrounds or other groups facing discrimination.[25]

6.3 Common Mental Health Conditions in Adolescents

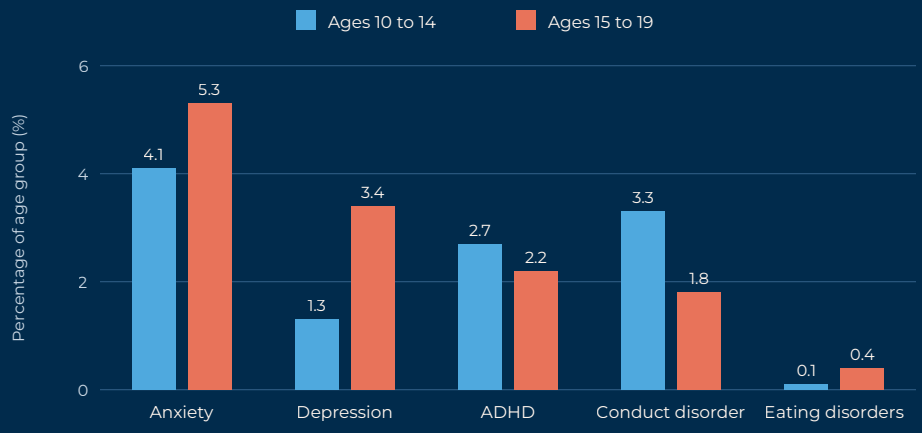


Figure 5: Estimated share of adolescents with selected conditions. Data from the WHO fact sheet "Mental health of adolescents" (2025), citing the Global Burden of Disease 2021 database. Chart made for this guide.

- **Emotional disorders.** Anxiety disorders are the most common conditions in this age group and are more common among older adolescents. Depression and anxiety can affect school attendance and schoolwork, and social withdrawal can increase loneliness.[26]

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24. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

25. Ibid. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

26. WHO, Mental health of adolescents, fact sheet (2025), section on emotional disorders. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

- **Behavioural disorders.** ADHD and conduct disorder are more common among younger adolescents and can affect education.[27]
- **Eating disorders.** These commonly begin in adolescence or young adulthood, affect girls more often than boys, and often occur together with depression, anxiety and substance use disorders.[28]
- **Psychosis.** Conditions with symptoms of psychosis most commonly begin in late adolescence or early adulthood.[29]

6.4 Suicide and Self-Harm

> **Handled With Care**
>
> This section is short on purpose. It gives you the facts you need for debate without unnecessary detail. If this topic affects you personally, please speak to a trusted adult today.

WHO estimates that **727,000 people** (of all ages) died by suicide in 2021.[30] UNICEF reported in 2021 that almost **46,000** children and adolescents aged 10 to 19 die by suicide every year.[31] WHO lists several risk factors, including harmful use of alcohol, abuse in childhood, stigma against seeking help, barriers to accessing care, and access to means. WHO also notes that digital media can either strengthen or weaken suicide prevention efforts.[32] The key message for this committee is that **suicide is preventable**, and WHO has published a guide, *LIVE LIFE*, to help countries with suicide prevention.[33]

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27. Ibid., section on behavioural disorders. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

28. Ibid., section on eating disorders. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

29. Ibid., section on psychosis. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

30. World Health Organization, WHO releases new reports and estimates highlighting urgent gaps in mental health, news item (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-who-releases-new-reports-and-estimates-highlighting-urgent-gaps-in-mental-health

31. UNICEF, The State of the World's Children 2021: On My Mind (October 2021). Available at: https://data.unicef.org/resources/sowc-2021/

32. WHO, Mental health of adolescents, fact sheet (2025), section on suicide and self-harm. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

33. World Health Organization, LIVE LIFE: an implementation guide for suicide prevention in countries (2021). Available at: https://www.who.int/publications/i/item/9789240026629

7. Understanding the Agenda, Part B: Addiction and Substance Use

WHO states that young people are "especially vulnerable to developing harmful substance use patterns that can persist across the lifespan."[34] In other words, habits that start at 14 can last for a lifetime. Let us look at each part of the problem.

7.1 Alcohol

Alcohol is the most widely used substance among young people in many regions. In 2019, **23.5%** of all 15 to 19 year olds worldwide were current drinkers. The rates were highest in the WHO European Region (**45.9%**) and the Region of the Americas (**43.9%**).[35]

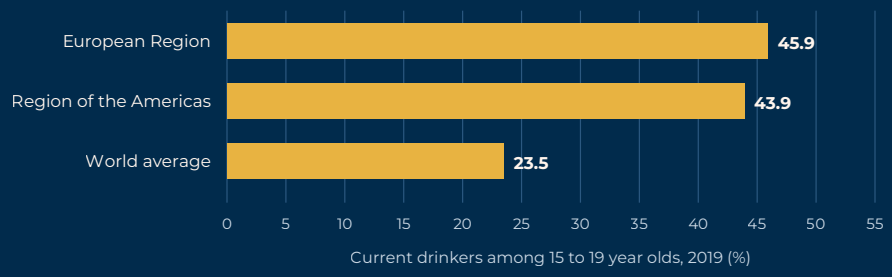


Figure 6: Data from WHO's Global status report on alcohol and health and treatment of substance use disorders (2024), as summarised in the WHO news release of 25 June 2024. Chart made for this guide.

Alcohol harms are serious. WHO reports that **2.6 million deaths** in 2019 were caused by alcohol, which is 4.7% of all deaths, and that the highest proportion of alcohol-attributable deaths (13%) was among young people aged 20 to 39.[36] WHO's main tools here are the **Global Alcohol Action Plan 2022 to 2030** and the **SAFER** package of policies.[37]

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34. WHO, Mental health of adolescents, fact sheet (2025), section on risk-taking behaviours. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

35. WHO, Over 3 million annual deaths due to alcohol and drug use, majority among men, news release (25 June 2024). Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

36. Ibid. Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

37. World Health Organization, SAFER initiative. Available at: https://www.who.int/initiatives/SAFER

7.2 Tobacco, Nicotine and E-Cigarettes



At least 15 million children aged 13 to 15 use e-cigarettes
(each figure represents 1 million children)

Figure 7: Based on WHO's first global estimate of e-cigarette use, news release of 6 October 2025. Pictogram made for this guide.

In October 2025, WHO published its first global estimate of e-cigarette use. It found more than 100 million people vaping worldwide, including at least 86 million adults and at least 15 million children aged 13 to 15.[38] At the same time, the same report showed that tobacco use overall has fallen: since 2010 the number of tobacco users dropped by 120 million.[39] So the picture is mixed: progress on cigarettes, new concerns about newer nicotine products. WHO also notes that many adult smokers had their first cigarette before the age of 18.[40]

7.3 Cannabis and Other Drugs

WHO reports that in 2022, the prevalence of cannabis use among adolescents was **higher** than among adults globally (5.5% compared with 4.4%).[41] The United Nations Office on Drugs and Crime (UNODC), which works closely with WHO on drug prevention, found that on average young people around the world use drugs at least as much as adults.[42]

At the launch of the *World Drug Report 2026*, UNODC researchers estimated that **63 million people** suffer from drug use disorders and that only about **8%** of them have access to drug treatment.[43] UNODC also reported that young people are increasingly seeing cannabis as harmful, which is an important change in long-term trends.[44]

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38. WHO, WHO tobacco trends report: 1 in 5 adults still addicted to tobacco, news release (6 October 2025). Available at: https://www.who.int/news/item/06-10-2025-who-tobacco-trends-report-1-in-5-adults-still-addicted-to-tobacco

39. Ibid. Available at: https://www.who.int/news/item/06-10-2025-who-tobacco-trends-report-1-in-5-adults-still-addicted-to-tobacco

40. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

41. WHO, Mental health of adolescents, fact sheet (2025), citing UNODC World Drug Report 2024. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

42. United Nations Office on Drugs and Crime, World Drug Report 2025, Key findings (June 2025). Available at: https://www.unodc.org/unodc/en/data-and-analysis/world-drug-report-2025.html

43. United Nations, Commission on Narcotic Drugs special event presenting the UNODC World Drug Report 2026, official transcript (26 June 2026). Available at: https://transcripts.un.org/en/asset/k19/k19rfgzd3w

44. Ibid. Available at: https://transcripts.un.org/en/asset/k19/k19rfgzd3w

7.4 Gaming and Social Media: "Behavioural" Addictions

Not every addiction involves a substance. WHO includes **gaming disorder** in the 11th revision of its International Classification of Diseases (ICD-11). It is defined as a pattern of gaming with impaired control, gaming taking priority over other interests and daily activities, and continuing to game despite negative consequences. Importantly, WHO adds that studies suggest gaming disorder affects only a small proportion of people who play digital or video games.[45]

Social media use has also become part of this discussion. The Health Behaviour in School-aged Children (HBSC) study, run with the WHO Regional Office for Europe, surveyed almost 280,000 young people aged 11, 13 and 15 in 44 countries and regions in Europe, Central Asia and Canada in 2022.[46]

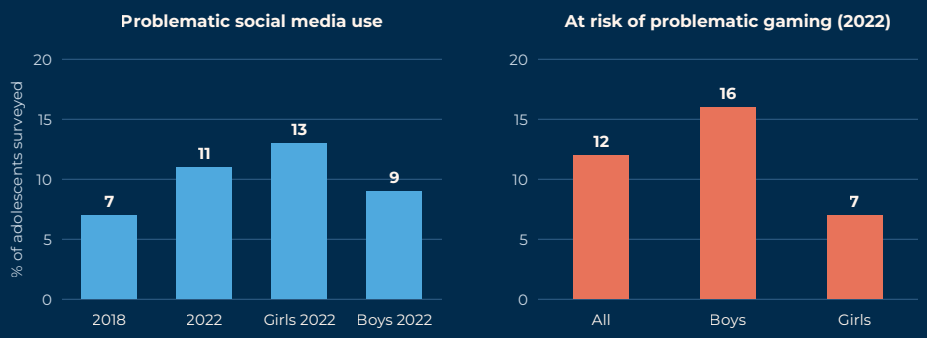


Figure 8: Data from the HBSC study as reported by the WHO Regional Office for Europe (2024). Covers Europe, Central Asia and Canada only, not the whole world. Charts made for this guide.

Key findings: problematic social media use rose from **7% in 2018 to 11% in 2022**, with girls reporting higher levels than boys (13% compared with 9%), and **12%** of adolescents were at risk of problematic gaming, with boys more affected than girls (16% compared with 7%).[47] The same report also found something positive: adolescents who were heavy but non-problematic users reported stronger peer support and social connections.[48] Technology is not simply "good" or "bad"; how it is used matters.

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45. WHO, Gaming disorder: frequently asked questions. Available at: https://www.who.int/standards/classifications/frequently-asked-questions/gaming-disorder

46. WHO Regional Office for Europe, Teens, screens and mental health, news release (25 September 2024). Available at: https://www.who.int/europe/news-room/25-09-2024-teens--screens-and-mental-health

47. WHO Regional Office for Europe, Teens, screens and mental health (2024). Available at: https://www.who.int/europe/news-room/25-09-2024-teens--screens-and-mental-health

48. Ibid. Available at: https://www.who.int/europe/news-room/25-09-2024-teens--screens-and-mental-health

Myth or Fact?

"Everyone who plays video games a lot has gaming disorder."
MYTH. WHO says studies suggest gaming disorder affects only a small proportion of people who play games.[49]

"Young people never use drugs as much as adults."
MYTH. WHO reports that adolescent cannabis use was higher than adult use in 2022.[50]

"Mental health is part of health."
FACT. The WHO Constitution has included mental well-being in its definition of health since 1946.[51]

8. Understanding the Agenda, Part C: Why Many Young People Do Not Get Help

Knowing there is a problem is only the first step. Many young people who need help never receive it. Here are the main reasons identified in official reports.

8.1 Not Enough Money for Mental Health

WHO's *Mental Health Atlas 2024* found that the median government spending on mental health is only about **2% of total health budgets**, a share unchanged since 2017.[52]

8.2 Not Enough Trained People

WHO reports a global median of about **13 mental health workers for every 100,000 people**, with severe shortages in low- and middle-income countries.[53] UNICEF found that in high-income countries the median number of child and adolescent

49. WHO, Gaming disorder: frequently asked questions. Available at: https://www.who.int/standards/classifications/frequently-asked-questions/gaming-disorder

50. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

51. Constitution of the World Health Organization, Preamble. Available at: https://www.who.int/about/governance/constitution

52. WHO, WHO releases new reports and estimates highlighting urgent gaps in mental health (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-who-releases-new-reports-and-estimates-highlighting-urgent-gaps-in-mental-health

53. WHO, Over a billion people living with mental health conditions: services require urgent scale-up (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up

psychiatrists was about 5.5 per 100,000 children, compared with about 0.1 in other countries.[54]

8.3 Very Low Treatment Coverage for Substance Use

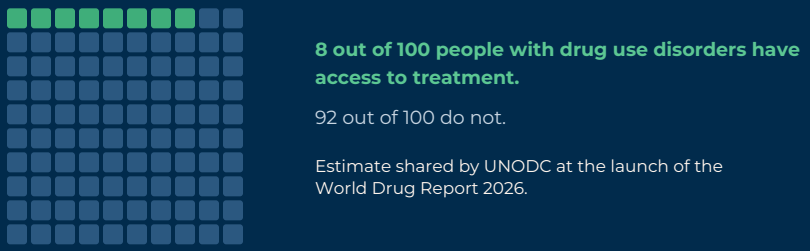


Figure 9: Each square represents 1% of the estimated 63 million people with drug use disorders. Source: UN official transcript of the World Drug Report 2026 launch (26 June 2026). Chart made for this guide.

For alcohol and drug use disorders together, WHO found that the share of people in contact with treatment services ranged from **less than 1% to no more than 35%** in 2019 in countries that provided data. Most of the 145 reporting countries had no specific budget line for this treatment, and almost half did not offer peer support groups.[55]

8.4 Stigma and Silence

WHO explains that stigma, discrimination and misconceptions about whether treatment works contribute to the treatment gap.[56] UNICEF's 2021 report called on societies to "break the silence" around mental health by addressing stigma and taking young people's experiences seriously.[57]

8.5 Gaps Between Rich and Poor Countries

WHO reports that in low-income settings fewer than 10% of people with psychosis receive treatment, compared with over 50% in higher-income countries.[58] There is

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54. UNICEF, The State of the World's Children 2021: On My Mind. Available at: https://data.unicef.org/resources/sowc-2021/

55. WHO, Over 3 million annual deaths due to alcohol and drug use, majority among men (25 June 2024). Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

56. Ibid. Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

57. UNICEF, The State of the World's Children 2021: On My Mind. Available at: https://data.unicef.org/resources/sowc-2021/

58. WHO, Over a billion people living with mental health conditions: services require urgent scale-up (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up

some good news too: over 80% of countries now include mental health and psychosocial support in emergency responses, up from 39% in 2020.[59]

Think About It

- If a country has very few psychiatrists, who else could be trained to help young people? Teachers? Nurses? Trained peers?
- How can a country reduce stigma so that a 13 year old feels comfortable asking for help?
- Is it fair to expect low-income countries to spend more on mental health when they also face other urgent health needs? How could the international community help?

59. Ibid. Available at: https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up

9. What the World Has Already Done

A strong resolution does not start from zero. It builds on what already exists. Here are the major international steps related to our agenda.

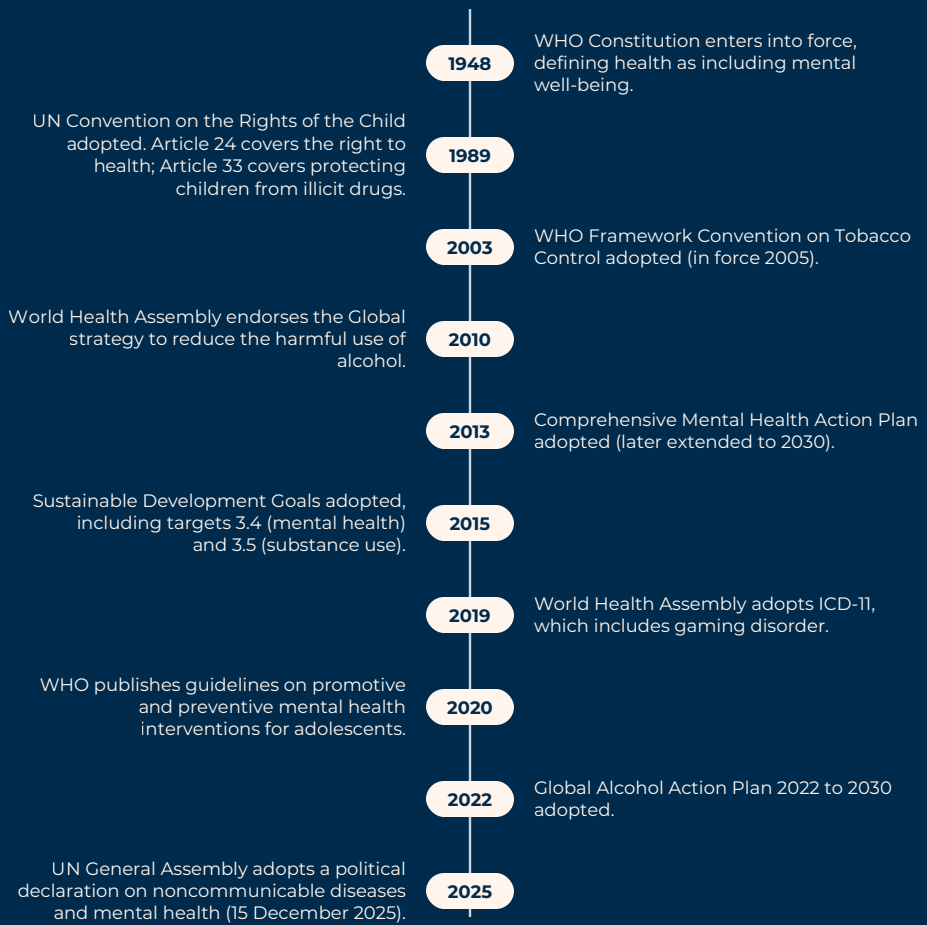


Figure 10: Timeline of key international milestones. Sources for each milestone are cited in the footnotes of this guide. Timeline made for this guide.

9.1 Key Frameworks Explained Simply

Convention on the Rights of the Child (1989). This UN treaty recognises every child's right to the highest attainable standard of health (Article 24) and asks States to take all appropriate measures, including educational ones, to protect children from the illicit use of drugs (Article 33).[60]

60. United Nations Office of the High Commissioner for Human Rights, Convention on the Rights of the Child. Available at: https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-child

Comprehensive Mental Health Action Plan 2013 to 2030. WHO's main global plan for mental health. It focuses on leadership, community-based services, promotion and prevention, and better information and research.[61]

Sustainable Development Goal 3. Target 3.4 includes promoting mental health and well-being, and target 3.5 calls on countries to "strengthen the prevention and treatment of substance abuse, including narcotic drug abuse and harmful use of alcohol."[62,63]

Alcohol: from the 2010 Global strategy to the Global Alcohol Action Plan 2022 to 2030. The Global Alcohol Action Plan 2022 to 2030 guides countries in implementing the 2010 WHO Global strategy to reduce the harmful use of alcohol, with a focus on the SAFER package of policies.[64]

ICD-11 and gaming disorder. The International Classification of Diseases is the global standard WHO uses to name and count health conditions. Its 11th revision (ICD-11), released by WHO in 2018 and adopted by the World Health Assembly in 2019, includes gaming disorder. WHO explains that including a condition in ICD is something countries consider when planning public health strategies.[65]

Helping Adolescents Thrive (HAT). A joint WHO and UNICEF initiative to strengthen policies and programmes that promote adolescent mental health, prevent mental health conditions, and help prevent self-harm and risky behaviours such as harmful use of alcohol and drugs.[66]

WHO guidelines on promotive and preventive mental health interventions for adolescents (2020). Evidence-based recommendations on helping adolescents build skills to manage emotions and stress.[67]

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61. World Health Organization, Comprehensive Mental Health Action Plan 2013 to 2030 (2021). Available at: https://www.who.int/publications/i/item/9789240031029

62. United Nations, Sustainable Development Goal 3: Good health and well-being. Available at: https://sdgs.un.org/goals/goal3

63. WHO, news release of 25 June 2024, notes to the editor. Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

64. WHO, news release of 25 June 2024, notes to the editor. Available at: https://www.who.int/news/item/25-06-2024-over-3-million-annual-deaths-due-to-alcohol-and-drug-use-majority-among-men

65. WHO, Gaming disorder: frequently asked questions. Available at: https://www.who.int/standards/classifications/frequently-asked-questions/gaming-disorder

66. WHO, Mental health of adolescents, fact sheet (2025), WHO response section. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

67. World Health Organization, Guidelines on mental health promotive and preventive interventions for adolescents (2020). Available at: https://www.who.int/publications/i/item/9789240011854

mhGAP Intervention Guide 2.0. A WHO guide that helps non-specialist health workers, such as nurses and general doctors, to assess and manage mental health conditions, including in children and adolescents.[68]

Mental health in schools. WHO's Regional Office for the Eastern Mediterranean developed a training package for educators to promote and protect mental health in schools.[69]

2025 Political Declaration on NCDs and Mental Health. For the first time, world leaders adopted one declaration covering both noncommunicable diseases and mental health. It includes a 2030 target of **150 million more people with access to mental health care**.[70] The UN General Assembly adopted it by a recorded vote of 175 in favour, 2 against and 1 abstention.[71] Delegates should check how their own country voted and why.

10. The Big Debates: Different Perspectives

This is the most important section for your debate. On each issue below, countries hold genuinely different views, usually because they have different laws, cultures, resources and experiences. **The Executive Board does not support any one side.** Your job is to find out which view your country holds and defend it well, while staying open to compromise.

10.1 Debate 1: Prevention First or Treatment First?

Some argue...	Others argue...
Prevention is cheaper and protects more people. Programmes in schools and communities can reach large numbers of young people before problems start.	Young people who are already struggling need help now. Prevention takes years to show results, while untreated conditions can become more serious.

68. World Health Organization, mhGAP Intervention Guide, version 2.0 (2016). Available at: https://www.who.int/publications/i/item/9789241549790

69. WHO Regional Office for the Eastern Mediterranean, Mental health in schools: a manual. Available at: https://applications.emro.who.int/docs/9789290225652-eng.pdf

70. World Health Organization, World leaders adopt a historic global declaration on noncommunicable diseases and mental health (16 December 2025). Available at: https://www.who.int/news/item/16-12-2025-world-leaders-adopt-a-historic-global-declaration-on-noncommunicable-diseases-and-mental-health

71. United Nations Digital Library, resolution A/RES/80/117, voting record. Available at: https://digitallibrary.un.org/record/4097012

Useful fact: WHO's own approach includes both promotion and prevention *and* early detection and treatment.[72]

10.2 Debate 2: How Should Countries Respond to Young People Who Use Drugs?

Some countries emphasise...	Other countries emphasise...
Strict laws and strong enforcement, to reduce the supply of drugs and send a clear message that drug use is not acceptable.	Health-centred responses, such as education, counselling and treatment, so that young people are not afraid to seek help.

Useful fact: the international drug control treaties and the Convention on the Rights of the Child both apply to this question. UNODC and WHO have jointly published *International Standards on Drug Use Prevention* and *International Standards for the Treatment of Drug Use Disorders*.[73] Many countries combine elements of both approaches. Find out where yours stands.

10.3 Debate 3: E-Cigarettes and New Nicotine Products

Some argue...	Others argue...
E-cigarettes should be banned or tightly restricted (for example, flavours and advertising), because so many children are using them.	E-cigarettes may help some adult smokers switch away from cigarettes, so rules should focus on keeping them away from young people rather than banning them for everyone.

Useful fact: WHO has called on governments to close regulatory gaps that allow the industry to target children and to extend tobacco control measures to new nicotine products.[74] Countries have chosen very different laws on this issue.

72. WHO, Mental health of adolescents, fact sheet (2025). Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

73. United Nations Office on Drugs and Crime, call for proposals referencing the UNODC-WHO International Standards (July 2024). Available at: https://unodc.org/unodc/drug-prevention-and-treatment/news-and-events/2024/july/call-for-proposals_-child-implementation-grants-programme-for-children-exposed-to-drugs-at-a-very-young-age-or-at-risk-of-using-drugs.html

74. WHO, WHO tobacco trends report, news release (6 October 2025). Available at: https://www.who.int/news/item/06-10-2025-who-tobacco-trends-report-1-in-5-adults-still-addicted-to-tobacco

10.4 Debate 4: Social Media and Gaming: Rules or Skills?

Some argue...	Others argue...
Governments should set minimum ages and hold technology companies responsible for protecting children online.	Teaching digital literacy, online safety and healthy habits works better, because bans are hard to enforce and online spaces can also offer real support and friendship.

Useful facts: Australia's social media minimum age law took effect on 10 December 2025, requiring age-restricted platforms to take reasonable steps to stop Australians under 16 from having accounts. The law includes an independent review within two years.[75] Meanwhile, the WHO Regional Office for Europe has recommended evidence-based school programmes covering responsible social media use, online safety, critical thinking and healthy gaming habits.[76] These two approaches are not necessarily opposites. Some countries may choose both.

10.5 Debate 5: Who Should Pay, and Who Should Act?

Some argue...	Others argue...
Each country is responsible for its own health system. National governments know their own people, cultures and needs best.	Low-income countries need international funding and technical help, because the gap in services between rich and poor countries is very large.

Useful fact: WHO data shows large differences in treatment between low-income and higher-income settings.[77]

10.6 Debate 6: Medicine, Therapy or Community?

WHO advises avoiding institutionalisation and over-medicalisation, prioritising non-medicine (non-pharmacological) approaches, and respecting children's rights for adolescents with mental health conditions.[78] At the same time, countries differ in

75. Australian Government, eSafety Commissioner, Social media age restrictions. Available at: https://www.esafety.gov.au/social-media-age-restrictions

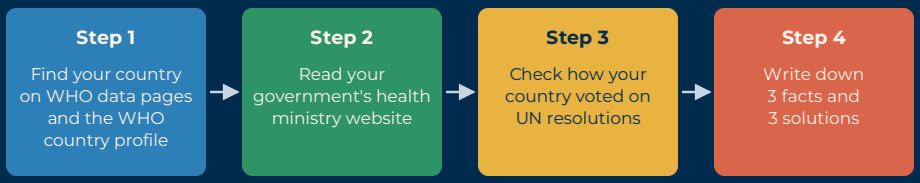
76. WHO Regional Office for Europe, Teens, screens and mental health (2024). Available at: https://www.who.int/europe/news-room/25-09-2024-teens--screens-and-mental-health

77. WHO, Over a billion people living with mental health conditions (2 September 2025). Available at: https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up

78. WHO, Mental health of adolescents, fact sheet (2025), section on early detection and treatment. Available at: https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health

how they organise care, the role of families and communities, and how culture shapes the way people talk about mental health. Think about what a respectful, culturally appropriate system would look like in your country.

11. How to Research Your Country's Position



11.1 Research Checklist

- Does my country have a national mental health policy or plan? Does it mention children and adolescents?
- What are my country's laws on the minimum age for buying alcohol and tobacco? Does it regulate e-cigarettes?
- What does my country do about drugs: what are its main laws and programmes?
- Does my country have any rules about children and social media or online gaming?
- Does my country have school mental health or counselling programmes?
- How did my country vote on the 2025 Political Declaration on NCDs and Mental Health?
- Is my country a Party to the WHO Framework Convention on Tobacco Control?

11.2 Writing Your Position Paper

A position paper is usually one to two pages. A simple structure is:

1. **Introduction:** why this issue matters to the world and to your country.
2. **Your country's situation:** key facts and what your government already does (with sources!).
3. **Your country's position:** what your country believes should happen.
4. **Proposed solutions:** two or three realistic ideas that fit WHO's mandate.

Golden Rules for Delegates

- **Stay in character:** speak as your country, not as yourself.
- **Every number needs a source.** Official sources (WHO, UNICEF, UNODC, UN, your government) are best.
- **Be realistic.** WHO recommends; it does not command.
- **Be kind.** You are discussing real people's lives.

11.3 Further Reading

All of the following are official publications or pages from the United Nations system or national governments. They are listed by organisation so you can find them easily.

World Health Organization

1. Constitution of the World Health Organization. https://www.who.int/about/governance/constitution
2. Mental health of adolescents, fact sheet (2025). https://www.who.int/news-room/fact-sheets/detail/adolescent-mental-health
3. Mental disorders, fact sheet. https://www.who.int/news-room/fact-sheets/detail/mental-disorders
4. Suicide, fact sheet. https://www.who.int/news-room/fact-sheets/detail/suicide
5. Alcohol, fact sheet. https://www.who.int/news-room/fact-sheets/detail/alcohol
6. Comprehensive Mental Health Action Plan 2013 to 2030. https://www.who.int/publications/i/item/9789240031029
7. Guidelines on mental health promotive and preventive interventions for adolescents (2020). https://www.who.int/publications/i/item/9789240011854
8. mhGAP Intervention Guide, version 2.0. https://www.who.int/publications/i/item/9789241549790
9. LIVE LIFE: an implementation guide for suicide prevention in countries. https://www.who.int/publications/i/item/9789240026629
10. Global status report on alcohol and health and treatment of substance use disorders (2024). https://www.who.int/publications/i/item/9789240096745
11. SAFER initiative on alcohol. https://www.who.int/initiatives/SAFER
12. Gaming disorder: frequently asked questions. https://www.who.int/standards/classifications/frequently-asked-questions/gaming-disorder
13. WHO tobacco trends report news release (6 October 2025). https://www.who.int/news/item/06-10-2025-who-tobacco-trends-report-1-in-5-adults-still-addicted-to-tobacco

14. World mental health today and Mental Health Atlas 2024, news release (2 September 2025). https://www.who.int/news/item/02-09-2025-over-a-billion-people-living-with-mental-health-conditions-services-require-urgent-scale-up
15. Political declaration of the fourth high-level meeting on NCDs and mental health. https://www.who.int/publications/m/item/political-declaration-of-the-fourth-high-level-meeting-of-the-general-assembly-on-the-prevention-and-control-of-noncommunicable-diseases-and-the-promotion-of-mental-health-and-well-being
16. WHO Framework Convention on Tobacco Control. https://fctc.who.int/

WHO Regional Offices

1. WHO Europe: Teens, screens and mental health (2024). https://www.who.int/europe/news-room/25-09-2024-teens--screens-and-mental-health
2. HBSC data browser: social media. https://data-browser.hbsc.org/topics/social-media/
3. WHO Eastern Mediterranean: Mental health in schools, a manual. https://applications.emro.who.int/docs/9789290225652-eng.pdf

Other United Nations Sources

1. UNICEF, The State of the World's Children 2021: On My Mind. https://data.unicef.org/resources/sowc-2021/
2. UNODC, World Drug Report 2025. https://www.unodc.org/unodc/en/data-and-analysis/world-drug-report-2025.html
3. UN official transcript: presentation of the World Drug Report 2026. https://transcripts.un.org/en/asset/k19/k19rfgzd3w
4. UN Digital Library, resolution A/RES/80/117. https://digitallibrary.un.org/record/4097012
5. OHCHR, Convention on the Rights of the Child. https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-child
6. United Nations, Sustainable Development Goal 3. https://sdgs.un.org/goals/goal3

National Government Example

1. Australian Government eSafety Commissioner, Social media age restrictions. https://www.esafety.gov.au/social-media-age-restrictions

12. Questions a Resolution Must Answer (QARMA)

A good resolution should try to answer most of these questions. You do not need to answer all of them in one speech!

1. How can schools, families and communities be supported to protect young people's mental health?

2. How can countries with very few mental health workers expand care for young people, for example through trained teachers, nurses or community health workers?
3. What measures should be taken to stop the marketing of alcohol, tobacco and nicotine products to children, both offline and online?
4. How can young people who use substances be encouraged to seek help without fear or shame?
5. What is the right balance between protecting young people online and respecting their rights and freedoms?
6. How should countries collect better data on young people's mental health and substance use, especially in regions where data is missing?
7. How can the international community support low-income countries and humanitarian settings?
8. How can young people themselves be included in designing the solutions that affect them?
9. How will progress be measured, and who will report on it?

We look forward to seeing you in committee!

Syed Hudaifah | Pratheek Konda | Archith Ragi

Executive Board, World Health Organization, OakJMUN